

**Krishna Institute Of Medical Sciences Limited**1-8-31/1, Minister Road, Krishna Nagar Colony,  
Secunderabad**Phone/Fax:** 040-44885000/040-27840980**Email:** assistance@kimshospitals.com || **Website:**  
www.kimshospitals.com**DEPARTMENT OF INTERNAL MEDICINE/GENERAL MEDICINE  
DISCHARGE SUMMARY****PATIENT DETAILS**

|                                  |                                         |                                                     |
|----------------------------------|-----------------------------------------|-----------------------------------------------------|
| <b>Patient Name</b> :            | <b>Age/Gender</b> :                     | 24Y(s) 6M(s) 23D(s)/Male                            |
| VETAPALEM VENKATA NARENDRA KUMAR |                                         |                                                     |
| <b>IP No.</b> :                  | <b>Admn Date</b> :                      | IPSE2526046573 21-03-2026 10:17                     |
| <b>UMR No.</b> :                 | <b>Discharge Date</b> :                 | MRSE2425087325 25-03-2026                           |
| <b>Doctor Name</b> :             | <b>Specialization</b> :                 | DR. SHIVA RAJU.K INTERNAL MEDICINE/GENERAL MEDICINE |
| <b>Ward/Room/Bed</b> :           | <b>Mobile No.</b> :                     | TWIN SHARING/TWIN-12D/1291-A 9966288487             |
| <b>Address</b> :                 | 1-11-10/1/B, OLD COSTUMS BASTI BEGUMPET |                                                     |

**PRIMARY CONSULTANT****DR. SHIVA RAJU**MD(Internal Medicine) (GENERAL MEDICINE)  
Sr. Consultant Internal Medicine**SECONDARY DOCTOR****DIAGNOSIS**Hypertensive Emergency  
Nephritic syndrome**CHIEF COMPLAINTS**

Dizziness since morning 1day

**PRESENT HISTORY**

giddiness and C/o sweating No C/o blurring of vision and shortness of breath. C/o episodic palpitation. No c/o epigastric pain. No C/o fever, burning m micturition, vomiting and loose stools. patient admitted for further evaluation and management

**PAST HISTORY**

K/C/O Gout on Tab Febuget

**ON EXAMINATION**

PR : 136/min,  
BP : 200/110mmHg,  
SpO2 : 94% at RA  
CVS : S1S2+  
RS : BAE +  
P/A : Soft

## COURSE IN THE HOSPITAL

Patient presented to the hospital with the above-mentioned complaints, for which he was initially treated with tablet Nicardia 10 mg stat followed by 20 mg thrice daily, and hourly blood pressure monitoring was initiated. Investigations revealed increased 24-hour urinary protein of 1220 mg/day and urine protein-creatinine ratio of 1.3, with urine routine microscopy showing protein 3+; outside investigations showed hemoglobin 17.3 g/dL, total leukocyte count 9840 cells/mm<sup>3</sup>, platelet count 3.08 lakh/mm<sup>3</sup>, ESR 3 mm/hour, and HbA1c 5.9%, deranged lipid profile. 2D echocardiography showed concentric LVH, good LV systolic function with ejection fraction 60%, grade I LV diastolic dysfunction, trivial MR/TR, no pulmonary hypertension, no LV clot, and normal IVC collapsibility; ultrasound abdomen and pelvis revealed hepatomegaly with grade II fatty liver and bilateral grade II renal parenchymal changes. During the hospital course, the patient had no complaints suggestive of end-organ damage. Serum TSH (2.79 µIU/mL) and early morning cortisol (12.6 µg/dL) were within normal limits. Nephrology consultation (Dr. Sridhar Reddy) was obtained in view of proteinuria and hypertension, with impression of hypertensive urgency, young hypertension, and nephritic syndrome; the patient was advised tablet Nicardia, Telma, Met XL, and Cilnidipine, renal Doppler was normal, and further evaluation with renal biopsy was planned. Serum complement C3 was elevated at 205 mg/dL, and C4 was 48.8 mg/dL (within normal limits). Cardiology and ophthalmology consultations were obtained, with funduscopy revealing grade II hypertensive retinopathy. ENT consultation on examination revealed grade II tonsillar hypertrophy, gross deviated nasal septum to the left, and right inferior turbinate hypertrophy; sleep study was advised with further decision regarding renal biopsy on follow-up. Spot urine for VMA was negative. The patient had persistently elevated early morning blood pressures around 160/100 mmHg, which were managed with antihypertensives, and as of 25/03/2026, the patient is hemodynamically stable and is being discharged with advice to follow up on an OPD basis for further evaluation and management.

## CONDITION AT THE TIME OF DISCHARGE

Hemodynamically stable. Blood pressure controlled on medications. No acute complaints.

## DISCHARGE MEDICATIONS

| S.No | Drug | Dose/Qty | Route | Frequency | Food Order | Duration | Instructions |
|------|------|----------|-------|-----------|------------|----------|--------------|
|------|------|----------|-------|-----------|------------|----------|--------------|

## WHEN & HOW TO GET EMERGENCY CARE

If any emergency contact casualty 040- 71225100.

## FOLLOW UP

- Review with Dr. Shiva Raju from the INTERNAL MEDICINE/GENERAL MEDICINE on Tuesday, 31-03-2026

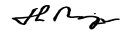
## DISCHARGE INSTRUCTIONS

- Tablet Telma H 80 mg once daily at 8:00 AM
- Tablet Met XL 25 mg once daily at 2:00 PM
- Tablet Arkamin 0.1 mg thrice daily if BP >140/80 mmHg
- Tablet Cilnidipine 10 mg twice daily
- Tablet Montelukast once daily at bedtime for 3 days

Regular blood pressure monitoring at home  
Maintain BP charting  
Reduce salt intake, increase physical activity  
Plan for sleep study on OPD basis  
Plan for renal biopsy at follow up  
Follow up after 7 days with BP chart and CUE report at Internal Medicine OPD

**NOTE**

Please Collect the pending lab reports at 2nd floor Lab Reception in a week's time

**Dr. Shiva Raju****Reg No: 42179**

MD (Internal Medicine) (GENERAL MEDICINE)

Sr. Consultant Internal Medicine

\*\* ICD Coding is based on International Classification of Diseases 11th Revision.

**ACKNOWLEDGEMENT:**

The Content of the Discharge Summary, Medications, Food & Drug Interactions, Care to be provided at Home, Nutrition & When and How to Obtain Emergency Care & etc. also have been Explained.

Signature of Patient / Attender: \_\_\_\_\_

**Started By** : Mamatha  
**Approved By** : Dr. Shiva Raju  
**Printed By** : Neeranka Arunkumar

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